

Spine AYUSH

Implementation Strategy & Architecture

Step-by-step guidance: from initial setup to full-scale operation

Ayuzee Hospital Management System

Integrative Spine Care · Ayurveda + Yoga + Modern Rehab

Owner: Chief Ayurvedic Physician / Super Admin

Platform: ayuzee.com

How to use this document

This guide is organized in 5 phases. Do NOT try to do everything at once. Complete each phase, run it with real patients for a week, then move to the next. Each phase builds on the one before. The single biggest risk is building wide instead of deep — resist adding new features until the current phase is battle-tested.

Prepared as an operating playbook for the practice owner and the platform admin (Dr Sindhu Roja). Review clinical content with a qualified physician before any patient acts on it. Software assists the clinician; it never replaces clinical judgment.

What This Covers

A practical, phased architecture for running a spine-focused AYUSH practice on Ayuzee HMS. It answers three questions in order: (1) What do I set up first? (2) How do I run daily operations reliably? (3) How do I grow and scale without breaking?

Phase	Focus	Outcome
0 — Foundation	Roles, branch, master data, safety	A safe, configured system
1 — Core Clinical	The one patient journey, end to end	Treat & track a real patient
2 — Retention & Ops	Follow-ups, packages, daily command center	No patient forgotten; cash tracked
3 — Growth	Referrals, content, offers, longevity	Predictable new-patient flow
4 — Scale & Impact	Franchise, rehab, CSR, integrations	Multi-center, mission-driven growth

The golden rule

Feature-complete is not the same as operationally ready. Value comes from connecting what you have to real patients — not from adding more screens.

Guiding Principles (read first)

- One patient, one flow: every patient moves through Assess -> Examine -> Protocol -> Treat -> Track. Master this loop before anything else.
- Safety before speed: the pre-treatment safety checklist gates Level-1 sessions. Never bypass it. Red-flag referral rules exist to protect the patient and you.
- Depth over width: a few modules used daily beat many modules used never. Turn features on only when the previous phase is stable.
- Clinician owns the record: AI/templates draft; the physician reviews and signs. Keep that line bright for liability and trust.
- Retention is the business: spine recovery needs 8-12 sessions. Drop-off after session 3 is where revenue leaks. Follow-ups are not optional.
- Measure outcomes honestly: VAS pain, ROM, functional scores before/after. Real data builds real testimonials and real referrals.

Phase 0 — Foundation & Safety Setup

Goal: a correctly configured, secure system before a single real patient is entered. This is the least glamorous phase and the most important.

0.1 People & Access

- Owner (you): Super Admin — full implementation power, owner-only Spine pages (Super Admin, Chief Physician).
- Admin (Dr Sindhu Roja): admin role — all HMS + Ayuzee modules for building real data; no super-admin/owner tier. (Already configured.)
- Add staff later with least-privilege roles: receptionist, therapist, pharmacist, lab_tech, nurse — each sees only their modules.

0.2 Branch & Clinic Profile

- Create the main branch: name, address, city, phone, WhatsApp, AYUSH license no., GSTIN, opening hours.
- Set the clinic's systems practiced and specialties (spine, panchakarma, etc.).
- This drives billing headers, geo-SEO pages, and patient-card details later.

0.3 Master Data & Safety Templates

- Seed / review the SOP & Safety checklists: pre-treatment safety, daily open/close, onboarding, PK pre-procedure.
- Review the seeded rehab protocols, therapy checkpoints, and drug-interaction data with a physician before use.
- Confirm the pre-treatment safety gate is active on the Level-1 session recorder.

Phase 0 exit criteria

Roles assigned, branch created, safety checklists live, protocol content physician-reviewed. Security: enable leaked-password protection; lock down any views exposing patient data to anon before real data entry.

Phase 1 — Core Clinical Journey

Goal: take ONE real patient from first contact to tracked outcome. If this loop works smoothly, you have a clinic. Everything else is amplification.

The 5-step flow

Step	What happens	Where in Ayuzee
1. Assess	AI assessment / intake, chief complaint, history	Spine AYUSH -> Assessment
2. Examine	Posture, ROM, dosha, functional exam	Spine AYUSH -> Examination
3. Protocol	Build phased treatment protocol + care plan	Treatment Protocol Builder
4. Treat	Deliver Level-1/2 session; safety checklist first	Level 1 / Level 2 Session
5. Track	VAS, ROM, functional & recovery scores	Outcome Tracker

1.1 Daily use

- Use the Spine Command Center as the home screen — it shows the ONE next action per patient.
- Register the patient once; reuse across visits. Capture VAS pain at baseline (this is your outcome anchor).
- Record every session honestly, including adverse or minimal responses — that data protects you and improves protocols.

Phase 1 exit criteria

You have treated at least one real patient through all 5 steps, recorded baseline + post VAS, and can show a before/after outcome. Do this for 5-10 patients before Phase 2.

Phase 2 — Retention & Daily Operations

Goal: stop patients falling through the cracks and start tracking money. This is where a clinic becomes a sustainable practice.

2.1 Follow-up engine

- Set your follow-up rules once (Day-1 check-in, missed-appointment, mid-treatment, package-ending, 30-day maintenance, referral request).
- Use the 'Due Now' list daily. Until WhatsApp Business API is connected, use click-to-WhatsApp links to send in one tap.
- Result: +course completion, -drop-offs, more referrals.

2.2 Packages, memberships & cash

- Create subscription/membership plans and time-limited offers.
- Track package usage per patient; flag when 3 sessions remain (upsell moment).
- Connect Razorpay payment links so packages/memberships can actually be collected (Integrations page has the guide).

2.3 The daily command center

- Morning: review today's queue and follow-ups due.
- Per patient: one next action, one tap.
- End of day: close the loop — pending follow-ups sent, tomorrow planned.

Phase 2 exit criteria

Follow-ups going out daily (even if manual one-tap), at least one paid package/membership recorded, and a daily open/close routine in place.

Phase 3 — Growth & Demand Generation

Goal: a predictable flow of new patients that does not depend on you personally chasing leads.

3.1 Referral & influencer network

- Onboard patient advocates, ethical medico referrers, and influencers via the Referral Network Hub.
- Share the public sign-up page (/spine/refer). Enable ?ref= link auto-attribution so credit is never lost.
- Keep medico referrals ethical (recognition / reciprocal), not cash-per-head — aligns with India medical ethics.

3.2 Content & trust

- Publish success stories (with consent) — before/after VAS is powerful social proof.
- Use the Video Library (YouTube embeds), Social Hub, and Patient Library to educate and retain.
- Run offers/promotions for seasonal or condition-specific campaigns.

3.3 Longevity & retention products

- Wellness Age tracker and Rejuvenation (Rasayana) programs turn one-time patients into long-term members.
- Retreats / boot camps create high-value annual touchpoints.

Phase 3 exit criteria

At least one active referrer bringing attributed leads, 3+ published success stories, and one running offer or campaign.

Phase 4 — Scale, Rehab & Social Impact

Goal: expand beyond a single clinic and add mission-driven depth — carefully, only once Phases 1-3 are solid.

4.1 Multi-center / franchise

- Use the Franchise Hub: setup wizard, layout planner, operations dashboard, cost calculator, staff roster, patient portal.
- Each center gets least-privilege staff roles and its own branch data.

4.2 Spinal Injury Rehab (sub-acute / chronic adjunct)

- Enroll post-op / SCI / trauma / degenerative patients in the Rehab Registry with phased protocols.
- Tele-rehab follow-ups monitor the 7 complication red-flags (dysreflexia, pressure sore, UTI, new deficit, DVT, respiratory, wound infection) and flag referral needs.
- Guardrail: this is supportive/adjunct care, NOT acute emergency management.

4.3 CSR & public impact

- Track donor grants and patient sponsorships in the CSR Fund Manager.
- The public /spine/impact page shows anonymized aggregate impact for corporate donors — no patient PII.
- Compliance note: software tracks allocation/impact; fund receipt, 80G, CSR-1 and audit are handled by your CA/finance.

4.4 Integrations

- WhatsApp Business API for fully-automatic messaging.
- Razorpay for online payments.
- ABDM/ABHA for national health-record interoperability when ready.

Phase 4 exit criteria

A second center or rehab/CSR program running on real data, with integrations connected where external accounts exist.

Technical Architecture (reference)

For the admin/technical operator. The platform is a React + Vite + TypeScript front end on Netlify (ayuzee.com), backed by Supabase (Postgres + Auth + RLS).

Layer	Technology	Notes
Frontend	React + Vite + TypeScript	Deployed to Netlify (ayuzee.com)
Backend	Supabase Postgres	Row-Level Security on all tables
Auth	Supabase Auth	Roles in user_roles; HMS access via doctors row
Access model	admin / super_admin + hms_role	admin = all modules; super_admin = owner-only
Data safety	RLS owner + admin policies	Public data only via SECURITY DEFINER aggregate RPCs

Access tiers

- super_admin (owner): implementation power + owner-only Spine pages. Reserved for the chief physician.
- admin: all HMS + Ayuzee modules for daily operations and data building (Dr Sindhu Roja).

- Staff roles: least-privilege, module-scoped (receptionist, therapist, pharmacist, lab_tech, nurse).

Data privacy principle

Patient and donor records are owner/admin-scoped by RLS. Anything shown publicly (e.g. impact page) must come through an aggregate, PII-free RPC — never a direct table read.

Key Risks & Priorities

Priority	Action	Why it matters
1	Connect click-to-WhatsApp / WhatsApp API	Retention engine is inert without real messaging
2	Enable ?ref= lead auto-attribution	Referral credit is currently lost at intake
3	Connect Razorpay payment links	Packages/memberships can't be collected yet
4	Fix views exposing auth.users to anon	Patient-data exposure risk before real data
5	Enable leaked-password protection	Account security for admins with data access
6	Pilot 1 week with real patients	Reveals friction no feature-building can

Final expert advice

Freeze new features. Connect the last mile (messaging, payments, attribution). Harden security before real patient data. Then pilot with real patients for a week. That is what turns an impressive build into a working, defensible clinical business.